

monitored with the goal of prolonging the pregnancy as long as possible. The mother may be hospitalized or prescribed drugs to lower blood pressure and will probably be advised to rest as much as possible. Because preeclampsia can affect blood flow to the placenta, regular ultrasound and [Doppler scans](#) will be done to check the baby's growth and to look for signs of [placental insufficiency](#). If your doctor is worried that your blood pressure is dangerously high despite medication, you're losing a lot of protein in your urine, or there are anxieties about the baby, immediate delivery, such as an induction (see [Induction of labor](#)) or a [Cesarean](#), will be recommended.

Group B Streptococcus Ten to 30 percent of women carry group B Streptococcus (GBS) in their vagina, which is completely normal, and does not cause any symptoms. However, 1 in 2,000 women can pass GBS to their baby once the water is broken, and the baby may develop a severe GBS-related illness.

What might be done If a woman is known to carry GBS, and if there are risk factors such as prolonged rupture of the membranes, or the baby is premature, intravenous antibiotics may be recommended once in labor, which usually prevents further problems. Women are generally screened for GBS at 35–37 weeks. If you carry GBS, you'll be given IV antibiotics during delivery.

Labor complications

Premature labor The normal length of pregnancy is 37 to 42 weeks. A baby born before 37 weeks is called premature or preterm (see [Premature birth](#)).

Fetal distress During labor, the baby is monitored for signs of distress, which can indicate that the baby's oxygen supply is reduced. One sign of fetal distress is meconium-stained fluid. This alone doesn't always indicate fetal distress, but if it's combined with a slowing of the baby's heart rate, fetal distress is more likely and steps may be taken for a prompt delivery. If thick meconium is present, the baby could aspirate it at birth, causing breathing problems and lung infection.

Failure to progress Sometimes the cervix fails to dilate as expected during the first stage of labor. Several factors can hamper the progress of labor: the baby's head may be too large for the pelvis; there may be inefficient contractions; or the baby may not be in the right position (see [Lie and presentation](#)).

Cord prolapse Rarely, the umbilical cord lies below the baby. This is more likely in a breech birth, or where the baby lies in a transverse position. In these cases, when the water breaks, the cord can slip through the cervix. This is an emergency because the cord may be compressed and restrict or cut off the baby's oxygen supply.

What might be done If you have an umbilical cord prolapse, an emergency Cesarean is typically performed.

Shoulder dystocia is when the baby's head has delivered but the shoulders are stuck. It's more common if the baby is big or the mother has diabetes.

What might be done The nurse will call for additional help. The mother's legs will be lifted up to help the baby's shoulders down and an [episiotomy](#) may be done. If the baby still doesn't come easily, the doctor will do maneuvers to help release the shoulders and aid the delivery (see also [Shoulder dystocia](#)).

Postpartum hemorrhage is said to occur if a woman loses more than 2 pints (1,000 ml) of blood within 24 hours of birth. It can be due to the uterus not contracting quickly enough, to incomplete delivery of the placenta, or to vaginal tears. Active management in the delivery of the placenta decreases this risk. Factors that increase the risk include a large baby or twins; prolonged labor; or bleeding before the labor.